

Patient ID: PA04	Patient Name: Anonymous
Age: 52 Years	Sex: M
Accession Number:	Modality: MR
Referring Physician:	Study: MRI CERVICAL + DORSAL+ WSS
Study Date:	

MRI CERVICAL SPINE

PROTOCOL:

Axial T1 and T2 Field Echo

Sagittal T1 and T2 Fast Spin Echo

Coronal STIR

CLINICAL BRIEF:

- H/O DIFFCULTLY TO WEARING WEIGHT
- DIFFCULTLY TO WALK
- N/H/O DM HTN THYRIOD TB
- N/H/O TRAUMM A
- N/H.O RSA PARLYSED ATTACK

OBSERVATION:

Straightening of cervical curvature with mild reversal of cervical lordosis.

Degenerative changes are seen in the form of multilevel disc dessication changes and marginal osteophytes formation.

Posterior disc osteophyte complexes are seen at C3-4, C4-5, C5-6 and C6-7 levels indenting the ventral theca and narrowing the bilateral neural foramina. There is seen indentation of the cord at C3-4, C4-5 and C5-6 levels, however no abnormal signal intensity seen in the cord.

Few Schmorl's nodes are seen along the cervical levels.

Rest of the visualized vertebrae show normal height. No antero or posterolisthesis seen.

The cervico-medullary and cranio-vertebral junctions are unremarkable.

The ligamentum flavum and facetar articulations are unremarkable.

No pre or paravertebral collection seen.

Bony canal diameter at the level of cervical vertebre are as follows (mm)-

Level :	C2-C3	C3-C4	C4-C5	C5-C6	C6-C7
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(mm): 12.0 8.0 9.0 6.0 7.0

IMPRESSION:- MRI FEATURES ARE SUGGESTIVE OF:

- **CERVICAL SPONDYLOSIS.**
- **POSTERIOR DISC OSTEOPHYTE COMPLEXES AT C3-4, C4-5, C5-6 AND C6-7 LEVELS INDENTING THE VENTRAL THECA AND NARROWING THE BILATERAL NEURAL FORAMINA WITH INDENTATION OF THE CORD AT C3-4, C4-5 AND C5-6 LEVELS, HOWEVER NO ABNORMAL SIGNAL INTENSITY IN THE CORD.**

Please correlate clinically and with other investigations.